

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: The patient is at 32 weeks gestational age with a medium risk tier for preeclampsia and a medium risk tier for macrosomia. Key findings include an elevated systolic blood pressure (142 mmHg) and positive proteinuria (dipstick 1), which require clinical review for preeclampsia. For macrosomia risk, a high pre-pregnancy BMI (42.4) and borderline fasting glucose are noted. No severe preeclampsia symptoms are reported.

Patient Snapshot (Recent Inputs)

Parameter	Value	Notes
Blood Pressure	142/79 mmHg	Hypertensive threshold ≥140/90, severe ≥160/110.
Proteinuria (dipstick)	1	≥1+ supports PE suspicion in context of HTN.
Symptoms	Headache=0, Vision=0, RUQ Pain=0, Swelling=0	Severe features increase escalation priority.
Glucose	Fasting=99 mg/dL 1hr PP=126 mg/dL	Targets typically fasting <95, 1hr <140 (pregnancy goals).
HbA1c	5.6	Higher HbA1c indicates sustained glycemic elevation.
Maternal Context	Age=38 BMI=42.4 GDM=0	Risk modifiers for fetal overgrowth + metabolic risk.
Activity / Sleep	Post-meal walk=29 min Sleep=6.1 hrs	Lifestyle factors influence glycemic response.
Food Log	paneer salad	Used for dietary correlation patterns.

Reason for Alert

Action Requested: ALERT_DOCTOR

Clinical Rationale: Patient TEST_MED_MED, 32 weeks gestational age, has a medium preeclampsia risk tier. Key vitals include systolic BP 142 mmHg (elevated per hypertension in pregnancy guidelines) and proteinuria dipstick 1. Diastolic BP is 79 mmHg. No severe headache, vision changes, upper abdominal pain, or swelling of face/hands reported. Also a medium macrosomia risk tier due to high pre-pregnancy BMI (42.4) and a fasting glucose of 99 mg/dL. An immediate clinical review is recommended as per ACOG guidelines on hypertensive disorders in pregnancy.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

⚠ This report is generated by an AI decision-support system and does not replace clinical judgment. Always follow institutional protocols.